
CONSULT FOR AFRICA

Clearview: Discovery

Everything we need to ask, to ground the strategy in your real numbers rather than my assumptions

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Why this exists

The strategy document that accompanies this rests on assumptions about Clearview that only you can confirm. Chief among them: that your laboratory has spare capacity, that your conversion from enquiry to cycle is lower than you would like, that a meaningful share of your IVF pregnancies deliver somewhere other than your own hospital, and that patients are already travelling to you from outside Lekki. If those are wrong, the strategy changes, and I would rather find out now.

This is longer than a normal intake because you have two businesses and I would rather ask everything once. Rough numbers and honest estimates are fine throughout. We are looking for the shape of the group, not audited accounts.

An answer of "we do not track this" is itself useful information, and in this field it is usually the most useful answer of all, because it points directly at what is worth starting to measure. Please do not go and calculate anything. Where you would rather talk something through, write "discuss" and we will cover it in the working session.

Sections 3, 4 and 8 are the ones that decide the most. If you run out of time, do those.

1. The group as it stands

- 1.1 When did Clearview Fertility open, and when did Clearview Hospital open? Which came first?
- 1.2 In one or two sentences each, how would you describe what each business does today?
- 1.3 How many people work in the group, roughly, split between the two?
- 1.4 Are they one company or two? Same shareholders, or different?
- 1.5 If they are separate, how do money and work pass between them today? Does one invoice the other, and for what?
- 1.6 Which of the two makes money, and which is carried? Please be blunt.

2. Volume and seasonality

- 2.1 Roughly how many fresh IVF cycles do you do in a typical month?
- 2.2 What does a good month look like, and what does a bad month look like? Both in cycles.
- 2.3 Is there a seasonal pattern? For example, do December, the harmattan period, or the months after Christmas behave differently?
- 2.4 Has monthly cycle volume over the last three years been rising, flat, or falling?
- 2.5 How many frozen embryo transfers do you do in a typical month?
- 2.6 How many intrauterine insemination cycles, ovulation induction cycles, or other non-IVF fertility treatments?
- 2.7 On the hospital side, roughly how many deliveries a month, and how many caesarean sections?
- 2.8 Roughly how many gynaecological operations a month, and what kinds?

3. The funnel: what happens between someone hearing of you and cycling

This is the section I most want answered, and the one most likely to produce "we do not track this".



- 3.1 Roughly how many people contact Clearview Fertility in a month, in any way at all? Telephone, WhatsApp, Instagram, walk in, website.
- 3.2 Of those, roughly how many attend a first consultation?
- 3.3 Of those who consult, roughly what proportion ever go on to do a cycle with you?
- 3.4 How long, typically, from someone first contacting you to starting their first cycle? Days, weeks or months?
- 3.5 Who answers the telephone and the messages today, and is it their only job?
- 3.6 How quickly does an enquiry get a reply, honestly, on a busy week?
- 3.7 Is there a written price list or package a patient can be sent the same day? May we see it?
- 3.8 Do you follow up with people who consulted and did not proceed? If so, who does it and how?
- 3.9 What reason do people most often give for not going ahead?
- 3.10 Do you keep any record, anywhere, of enquiries that did not convert? A book, a spreadsheet, a phone?

4. Outcomes

This decides whether the strategy is available at all. A patient who is considering flying to you buys your outcomes, and nothing else.

- 4.1 Do you record clinical pregnancy rate per cycle? Per transfer?
- 4.2 Do you record **live birth** rate, as opposed to pregnancy rate? These are different numbers and the difference matters.
- 4.3 Can any of the above be broken down by the woman's age band? Under 35, 35 to 37, 38 to 40, 41 and over.
- 4.4 Where does this information live? A logbook, a spreadsheet, a clinical system, or in your head?
- 4.5 Do you know what happened to patients who conceived but delivered elsewhere? How do you find out?
- 4.6 What do you currently tell a new patient her chances are, and where does that figure come from?
- 4.7 What success rate, if any, does your marketing currently claim?
- 4.8 Do you audit your own laboratory performance, for example fertilisation rate, blastocyst rate, or survival after thawing?
- 4.9 Have you had a period where results were noticeably worse, and did you establish why?

5. Where your patients come from

The whole inbound thesis turns on this section.

- 5.1 Of your cycle patients, roughly what share live on the Lekki and Victoria Island axis?
- 5.2 What share come from mainland Lagos?
- 5.3 What share come from outside Lagos entirely? Which cities?
- 5.4 Have you ever treated a patient from another West African country? How did they find you?
- 5.5 Have you ever treated a patient living abroad, in the United Kingdom, United States, Canada or elsewhere? Roughly how many, and how did they find you?
- 5.6 For any patient who travelled to you, how did you handle the monitoring scans and blood tests? Did she relocate to Lagos for the whole cycle?



5.7 Did any travelling patient have a complication after going home? What happened?

5.8 Roughly what share of your patients come from: you personally, in your own clinic / another doctor referring / word of mouth from a previous patient / social media / the website / walk in / other?

5.9 How many doctors refer to you regularly? Could you name them?

5.10 Do those referring doctors get anything back? A report, a fee for work done, a thank you, nothing?

5.11 Do you currently advertise anywhere, and what do you spend a month?

6. The laboratory and the team

6.1 How many embryologists do you have?

6.2 How many of them can independently run a retrieval and fertilisation day, start to finish, without you or anyone else present?

6.3 What happens when your lead embryologist travels or is unwell? Has this happened, and what did it cost you?

6.4 How were they trained and how long have they been with you?

6.5 What is the maximum number of retrievals the laboratory could handle in a week if the patients existed? Your honest ceiling.

6.6 How many retrievals do you actually do in a typical week?

6.7 What equipment would need replacing or adding to raise that ceiling?

6.8 Do you have a witnessing system, electronic or manual, for identifying samples?

6.9 Who covers your clinical work when you are away?

6.10 Is there a second fertility doctor in the group, or is it you?

7. The tank: frozen embryos and storage

7.1 Roughly how many patients currently have embryos or eggs frozen with you?

7.2 How far back do the oldest stored embryos go?

7.3 Do you charge an annual storage fee? If so, how much, and what share of patients actually pay it?

7.4 How do you decide what happens to embryos when a patient stops paying or stops responding?

7.5 What does your consent form say about storage duration and disposal? May we see it?

7.6 Do you contact stored patients proactively to discuss a transfer or a second child? Who does that, and how often?

7.7 Of patients who had a baby with you and still have embryos stored, how many have come back for a sibling?

7.8 What proportion of your fresh cycles produce surplus embryos worth freezing?

8. Money

8.1 What does a fresh IVF cycle cost a patient at Clearview, all in? Please say clearly whether drugs are inside or outside that figure.

8.2 What does a frozen embryo transfer cost?



8.3 What do the add-ons cost: ICSI, embryo freezing, annual storage, assisted hatching, genetic testing if you offer it, donor eggs or sperm?

8.4 When did you last raise prices, and by how much?

8.5 What do stimulation drugs cost you for a typical cycle, and how do you buy them? Per patient, or in planned quantity?

8.6 Roughly what proportion of the price of a cycle goes out again as drugs and consumables?

8.7 How much of your input cost is effectively priced in dollars?

8.8 How do you handle it when the exchange rate moves sharply mid-cycle?

8.9 Do you offer instalments, a payment plan, or any financing? What proportion of patients use it, and do they pay?

8.10 Do any HMOs or corporates pay for anything you do? Which, and do they actually pay on time?

8.11 Do you offer any multi cycle or refund package today?

8.12 Roughly what are the monthly fixed costs of each business? Rent, salaries, power, security.

8.13 What is your rent position on both sites? Owned, leased, and if leased, when does it end?

8.14 Has either business ever been unable to meet a month's obligations? What happened?

9. The hospital, and what happens after the positive test

This is the section behind the largest number in the strategy document.

9.1 When a patient of Clearview Fertility becomes pregnant, what happens next, step by step?

9.2 At what point does she stop being your fertility patient?

9.3 Do you personally take over her antenatal care, or does someone else?

9.4 Roughly what proportion of your IVF pregnancies book their antenatal care at Clearview Hospital?

9.5 Roughly what proportion actually deliver at Clearview Hospital?

9.6 For those who go elsewhere, why? Do they choose to, or is it never offered?

9.7 Is there anybody whose job it is to make sure an IVF pregnancy books with you?

9.8 What can Clearview Hospital handle obstetrically? Caesarean section, twins, pre-eclampsia, preterm labour?

9.9 What neonatal capability do you have? Special care, incubators, ventilation, and to what gestation?

9.10 Where do you send a baby you cannot look after, and what does that cost the family?

9.11 IVF pregnancies carry a higher rate of twins, prematurity and caesarean section. Is the hospital currently equipped for the case mix your own clinic creates?

9.12 Do you offer anything after the birth? Postnatal care, paediatrics, immunisation, a six week check?

9.13 If a woman who conceived with you lives in Abuja or Accra, is there anything at all in place for her pregnancy?

10. Capacity, risk and what would break

10.1 If cycle volume doubled next month, what breaks first? Be specific.

10.2 How many consulting rooms, scan rooms and theatre sessions do you have available?



10.3 Is theatre availability for retrievals ever a constraint?

10.4 What is the longest wait a patient currently has between deciding to proceed and starting?

10.5 What worries you most about the group, at three in the morning?

10.6 What single thing, if it happened, would do the most damage to the business?

11. Regulation, governance and accreditation

11.1 What licences and registrations do the two businesses hold, and when do they renew?

11.2 Are you a member of any professional body for assisted reproduction, and does it audit you?

11.3 Do you have written clinical protocols, and when were they last reviewed?

11.4 Have you had any regulatory inspection, and what was the outcome?

11.5 Do you hold medical indemnity, and at what level?

11.6 Have you ever had a serious clinical incident or complaint in the fertility service? You may prefer to discuss this rather than write it.

11.7 If a patient in London asked you to prove your standards before flying out, what could you send her today?

11.8 Do you offer donor eggs or donor sperm? If so, how are donors recruited, screened, compensated and consented?

12. The competition

12.1 Which clinics do you lose patients to, and do you know why?

12.2 What do you think they do better than you?

12.3 What do you do better than them?

12.4 Do you know what they charge?

12.5 Has a patient ever told you she chose you over a named competitor? What was the reason?

12.6 Is there a clinic in Lagos you genuinely admire?

13. You, and what you actually want

This determines the engagement more than anything else in this document.

13.1 When you say you want stability and sustainability, which of these is closest to what you mean?

- The lumpy months frighten me and I want the troughs filled
- I am tired of being the only reason it works and I want to step back
- I want the group to be worth something to somebody else one day
- Something else, in your own words

13.2 Where do you want to be in five years, personally? Still operating, running the group, or out?

13.3 Is there a successor, a partner, or family in the business?

13.4 How many hours a week do you work, and how many would you like to?

13.5 Would you consider outside investment, or would you rather stay independent?

13.6 What have you tried already to fix the recruitment problem, and what happened?



13.7 Is there anything you have decided you will never do, commercially or clinically?

13.8 What would make you say, in two years, that this was worth it?

14. Practicalities

14.1 Who else should be in the room for the working session?

14.2 Is there anyone in the group we should not speak to yet, and why?

14.3 Would you be comfortable with us speaking to a few of your patients, with their consent?

14.4 Would you be comfortable with us speaking to two or three of your referring doctors?

14.5 What would you like to have documented that you do not have today?

14.6 When would you like to have a decision made by?

Please do not spend more than an hour on this. Where a question needs digging, leave it and mark it "discuss" so we can do that part together. The gaps tell us as much as the answers.